The Hillingdon Hospitals
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Maternity Triage and Telephone Triage

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Version	Author	Summary of Changes
4.0	Joanne Dunckley, Labour Ward Manager Paula Galea, Obstetric Lead for Labour Ward	Full review
4.1	Benjamin Viaris de Lesegno, Obstetric lead for DAU and Triage Felicity Bruce, Triage Lead Midwife	Minor change to Section 2.1. for telephone triage follow-up process. Author and Lead of guideline amended.

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1. Aim/Purpose

The aim of this guideline is to triage women/birthing people appropriately so that they see the right health professional in the right setting within an appropriate timeframe and receive the appropriate treatment which includes a follow-up where necessary, either over the telephone or through a face-to-face assessment.

2. The Guidance

Maternity Triage

Maternity triage is a multi-disciplinary-led assessment area. Triage provides a 24-hour service, 7 days a week for urgent midwifery and obstetric care. The service will see women/birthing people from 18+0 weeks gestation until 28 days postnatally. It is recommended women/birthing people call prior to attending on **01895 279054**. It is the emergency area for maternity at Hillingdon and whilst not recommended it is understood the woman/birthing person may present without calling in advance. All newborn babies requiring a review will be seen in the emergency department. The Midwifery Led unit (MLU) team will triage low risk women/birthing people in MLU if the service is available and transfer them to triage if any deviation arises from their low risk criteria.

2.1. Maternity Telephone Triage

All women/birthing people will be directed to the correct phone number on their hand-held notes for telephone calls related to unplanned antenatal and postnatal queries, including labour assessment. This will be to maternity triage or MLU telephone lines.
The call and the information given must be documented on the assessment/ call log sheet and transferred onto the electronic system. The telephone call is triaged by a midwife, who will then decide the subsequent management (Appendix 1). This should include a timeframe for individual attendance. The midwife should assess the women/birthing person directly over the phone, not via the partner or a family member. Women/birthing people who do not attend triage as advised should be contacted by the midwife at a time in keeping with the severity of the symptoms described, at the earliest 1h after the expected time and at the latest at the end of their shift. This phone call should be documented on the electronic system.

Options include (see Appendix 1):

- No further action following advice and reassurance from the midwife.
- Advised to be seen in the community by the general practitioner (GP)
- Advised to attend the urgent care centre (UCC) / ED emergency department.
- Contact the community midwife directly themselves for further support (in some circumstances the triage midwife should contact to community midwife to explain the need to attend the home).
- Assessment of the woman in maternity triage or in MLU.
- Admit directly to labour ward.

For phone calls outside of the triage guideline, appropriate referral should be made:

- If under 18+0/40 reporting vaginal bleeding or severe abdominal pain refer the woman/birthing person to ED.
- If the woman/birthing person is over 28 days since birth, they should be signposted to the GP or ED

London Ambulance Service (LS) follow their own PRE-ALERT criteria and will call directly to the Labour ward red phone to inform of arrival. On arrival it is the team's decision ie the on call obstetric registrar and/or labour ward co-ordinator if the woman/birthing person is to

remain on labour ward due to severity of presenting symptoms or de-escalate to triage or ED for ongoing care.

2.2. Roles and Responsibilities

Midwife

- Follows evidence-based pathways to provide consistent telephone advice to women/birthing people and other professionals who call the maternity triage.
- Refer women/birthing people with identified risk to appropriate place or health professional for review (see Appendix 1).
- Welcome women/birthing people who attend the maternity triage and MLU and conduct a face-to-face assessment.
- Initiate appropriate care pathway and time frame for women/birthing people to be seen (Appendix 2 for the traffic light flow charts)
- Complete the risk assessment form on the electronic system and update the electronic whiteboard including time frames, via the blue + symbol.
- Any samples sent to the laboratories for testing are noted in the triage results book.
- The midwife is responsible for checking sample results daily and (if necessary) informing the woman/birthing person of the results and sending out appropriate correspondence via ACCUREX or letter.
- All documentation must use the relevant care bundles relating to the algorithms, along-side auto-text if required.

Obstetrician

- Assess and review women/birthing people who present with risk factors that require obstetric input in maternity triage.
- Document clear plans of care for further follow up, discharge or admission as required.
- All obstetric doctors who are ST2 and below must discuss the proposed plan of care with the obstetric registrar or consultant for all in-patient admissions or if uncertain about the plan of care.
- Respond to GP queries.

Maternity Support Worker

- Assist the midwives in the running of maternity triage.
- Undertake clinical tasks as required.
- Welcome women/birthing people to maternity triage.
- Perform observations within the remit of their role, reporting findings to the midwife and document on the electronic system.
- Maintain stocks and general cleanliness of the clinical areas.
- Assist with administrative tasks such as answering calls and referring clinical calls to the midwife.

Administrative support / ward clerk

- Admit women/birthing people onto the electronic system.
- Record admissions in the admission book for maternity triage.
- Order ward stock.

2.3. Initial Antenatal / Postnatal Assessment

On arrival to triage the woman/birthing person is welcomed to the unit and admitted onto the electronic system. They will be seen by the midwife within 15 minutes of arrival and categorised based on their presenting symptoms. All clinical risk factors must be assessed.

This assessment includes:

- Maternal observations P / BP / RR / T / oxygen saturations (suitable to be completed by MSW)
- Urinalysis
- Clinical history – presenting complaint, current and past obstetric history
- Presence of normal fetal movements
- Abdominal palpation + SFH (SFH from 26+0 weeks)
- Fetal heart auscultation (over 18+0 weeks)
- Observation of any PV loss if reported

All findings should be documented on the electronic system and SFH plotted on growth chart in the hand-held notes.

Following on from this the woman/birthing person is categorised based on her main presenting complaint and the clinical findings to follow a pathway of care. The categories are:

- **Reduced fetal movements.**
- **Hypertension**
- **Abdominal Pain**
- **PV Bleeding**
- **Suspected Labour**
- **Spontaneous rupture of membranes**
- **Postnatal concerns**
- **Headaches**

At each assessment the midwife should also assess the women/birthing person's bladder sensation/ function either antenatally or postnatally as well as a review of their perineal wound if postnatal. If appropriate the midwife will then refer to our Perinatal Pelvic Health Service (PPHS) via email or referral booklet located in triage. For inclusion criteria for the PPHS referral please see their guideline - Perineal Tears and Repair guidelines

Each pathway has its own individual algorithm (see Appendix 2) which must be used to accurately prioritise the level of clinical urgency. Categorisation can be changed to a higher level (eg. yellow to amber) if a clinical condition changes. The pathway assigned to the woman/birthing person will determine how long they can wait for ongoing care.

2.4 Ongoing care and Obstetric Review (see Appendix 3)

Not every woman/birthing person attending triage will require an obstetric review. They should be given individualised care. The urgency of obstetric review is shown here:

- RED category = immediate transfer to labour ward and obstetric review

- ORANGE category = obstetric registrar review within 15-45mins of initial assessment

- YELLOW = obstetric SHO review with 2 hours of initial assessment

- GREEN = may be suitable for midwifery assessment and discharge

If the on call obstetric registrar cannot attend within the timeframes above, then the on-call gynaecology registrar should be contacted Twice daily 0830 and 2000, the Obstetric Handover on Labour Ward should include a discussion of the triage activity to promote a holistic view of the Maternity unit. This should include out of hours and weekends.

2.5 Category of Urgency

- **Red** category women/birthing people require immediate care by the multidisciplinary team and immediate transfer to Labour Ward. See Appendix 4 for quick reference guide. The list is not exhaustive and clinical judgement should be used.

- **Amber** category requires ongoing care by the midwife to be commenced immediately or within 15mins of initial triage assessment. The Amber scoring women/birthing people should be reviewed by the obstetric registrar ST3 and above by 45 minutes of assessment. Some criteria require obstetric review by 15 minutes, see Appendix 4.

-Yellow category women/birthing people can remain in the waiting area for ongoing care. The midwife should provide advice on how to escalate any concerns whilst waiting. Ongoing care such as blood tests or CTG must be commenced within 60 mins of initial triage assessment. The obstetric team (SHO or above) should review within 60 minutes of commencement of ongoing care.

If the only presenting symptom is RFM, then the woman/birthing person should have a CTG within 30 minutes (if other symptoms present such as PV bleeding or abdominal pain, CTG should be within 15 minutes). An Obstetric review may not be required for all cases of RFM only.

-Green category women/birthing people can remain in the waiting area for ongoing care. The midwife should provide advice on how to escalate any concerns whilst waiting. Ongoing care should be commenced within 4 hours of initial triage assessment. Obstetric review may not be required for this category but if needed, there should be a review by 4 hours from arrival.

2.6 Latent Phase of Labour / Established Labour

If symptoms and signs suggest that labour has started, and the woman/birthing person is >37+0 weeks, then the midwife should perform a vaginal examination.

Depending on findings a plan of care is implemented as per the guideline “Care in labour”. The woman/birthing person should be given the opportunity to discuss events and share their story. Care is planned in conjunction with their choices, for example care in the MLU depending on their chosen options for analgesia.

In the event of births outside the maternity unit, for example in the car park:

- A midwife will go from maternity triage and will ask for a second midwife from another area to assist, this is most likely to be from triage or labour ward where the care will be continued.
- The midwife will take the delivery equipment to the scene from labour ward or triage stock.
- Labour Ward will prepare a room. The woman/birthing person will also have the option to be taken to the MLU, if appropriate.
- A MSW will bring a wheelchair to help move the woman/birthing person to either the labour ward or MLU.

2.7 Multiple pregnancy Red Flags for twin-to-twin transfusion syndrome (TTTS)

TTTS occurs in approx. 1 in 6 monochorionic twin pregnancies (MCDA or MCMA), when there is unequal sharing of blood between the babies. To detect TTTS, women/birthing people with MC twins have scans every 2 weeks from 16 to 36 weeks. Any abnormal ultrasound changes are classified using Quintero criteria to assess the severity of TTTS). However rarely the problem can arise suddenly, with any of the following signs and symptoms. Red flags for TTTS include one or more of the following:

Symptoms reported by the mother/birthing person

- Sudden abdominal distension
- Abdominal pain
- Sudden breathlessness
- Inability to lie on their back.
- Reduced fetal movements.

Clinical signs

- Rapidly increased abdominal girth
- Inability to feel fetal parts on abdominal palpation.

Any woman/birthing person who has a multiple pregnancy with any of these red flags and presents to Maternity Triage needs to be seen by a registrar or consultant obstetrician.

2.8 Postnatal women/birthing people

Women/birthing people with urgent symptoms will be seen in maternity triage up to day 28 postnatally. Newborn babies with urgent clinical concerns will be seen in the emergency department.

When assessing a postnatal woman/birthing person either on the phone or in person, the midwives should consider risk factors for complications in recovery such as would infection:

- BMI
- Diabetes
- Malnutrition
- Comorbidities
- Smoking status
- Hb level
- Albumin level if known.

The community midwife should contact triage and refer in women/birthing people if they have concerns over their postnatal condition. Arrangements to attend triage will be made on an individual basis depending on the clinical circumstances.

2.9 Triage Escalation for high activity

The Maternity bleep holder supplies a SITREP form daily with 4 hourly intervals to the CSP (Clinical Site Practitioner). This will evidence the current activity within the Maternity Unit and raise areas of concern that may warrant further action. In addition, the Maternity bleep holder will be informed if the triage midwives have specific safety concerns about the activity levels within the area. Please see 'Maternity Escalation and Divert Policy Including Maternity Staffing' Guideline for reference to OPEL STATUS and Maternity Triage.

3. Associated Guidelines, Polices and Documents

- Care in labour
- Postnatal care
- Caring for women in Midwifery Led Birthing Unit
- Perineal Tears and Repair guidelines

4. Training Need Identified

No preceptor midwives to work in triage.

Training/Orientation package for all midwives who work in maternity triage.

5. Implementation and Monitoring

All guidelines, protocols and procedures are available on the intranet. All staff have a responsibility to check the Trust's intranet and ensure that they are referring to the most recent issue of the document. The weekly general information bulletin includes notification of publication of new/updated documents.

This clinical guideline, protocol or procedure will be monitored using local and National guidance and via the Trust Incident and Risk Management Policies. If risks/issues are identified further specific monitoring will be considered, for example, clinical audit or risk assessment.

6. References

The Sepsis Trust Sepsis six and red flags: Inpatient Maternal Sepsis Tool

MBRRACE (2021) Stillbirths and neonatal deaths in twin pregnancies

National Collaborating Centre for Women's and Children's Health (2011) *Hypertension in pregnancy: the management of hypertensive disorders in pregnancy.* Royal College of Obstetricians and Gynaecologists; London

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Royal College of Obstetricians and Gynaecologists (2023) *Maternity Triage (Good Practice Paper No. 17)*

Royal College of Obstetricians and Gynaecologists. (2012) Bacterial infection in pregnancy

7. Appendices

Appendix 1 – Referral Pathways of Care
Appendix 2 – Triage Flowcharts
Appendix 3 – Categories of Urgency and Timings
Appendix 4 – Red and Amber categories

Appendix 1: Referral Pathways of Care

TELEPHONE CALL TO MATERNITY TRIAGE
Midwife triages the call and records information on electronic system

Ask to attend Maternity Triage

Home or CMW BP of 140/90 or over with or without presence of proteinuria (AN) or
BP of 160/100 or over (PN)
Headaches – severe and/ or persistent
Women/birthing people in labour
Spontaneous rupture of membranes
Reduced fetal movements
PV bleeding
Abdominal pain
Women/birthing people who arrive by ambulance
Postnatal women/birthing people up to 28 days
DVT symptoms
Post USS requiring urgent follow up care if alongside triage algorithms

ED
Women/birthing people who have medical problems requiring urgent input such as possible PE

Make appointment for DAU

Known hypertension with ongoing care plan and no acute symptoms
Proteinuria without Hypertension
Itching with normal FM
Cervical sweeps
Blood tests

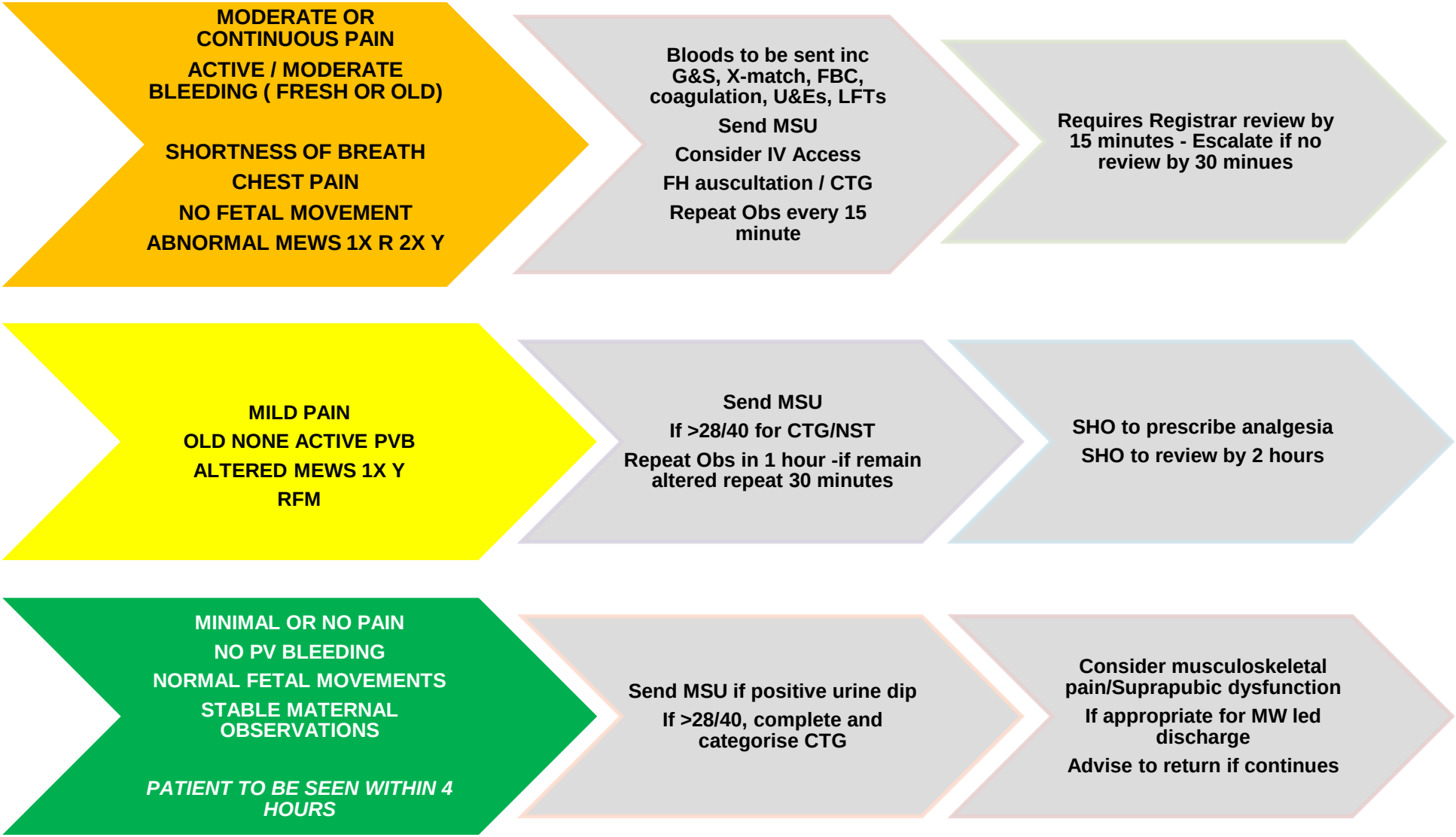
Scan reviews in asymptomatic cases
Emergency USS/
Small/ large for dates
Presentation
Suspected oligohydramnios

Refer to General Practitioner

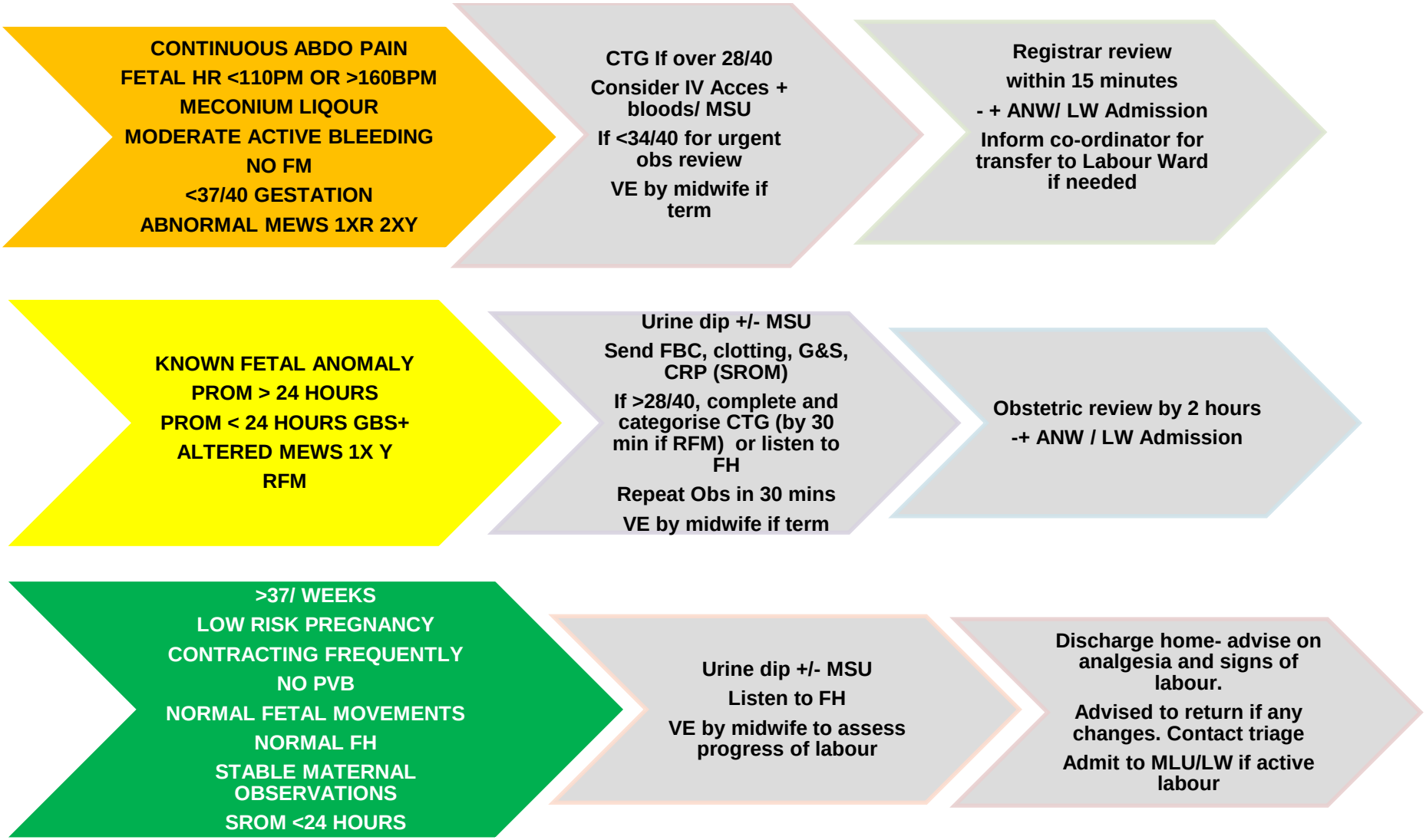
Minor ailments e.g. colds, headaches with no other symptoms, haemorrhoids. Minor aches and pains, diarrhoea and vomiting, suspected chicken pox

Appendix 2: Triage Flowcharts

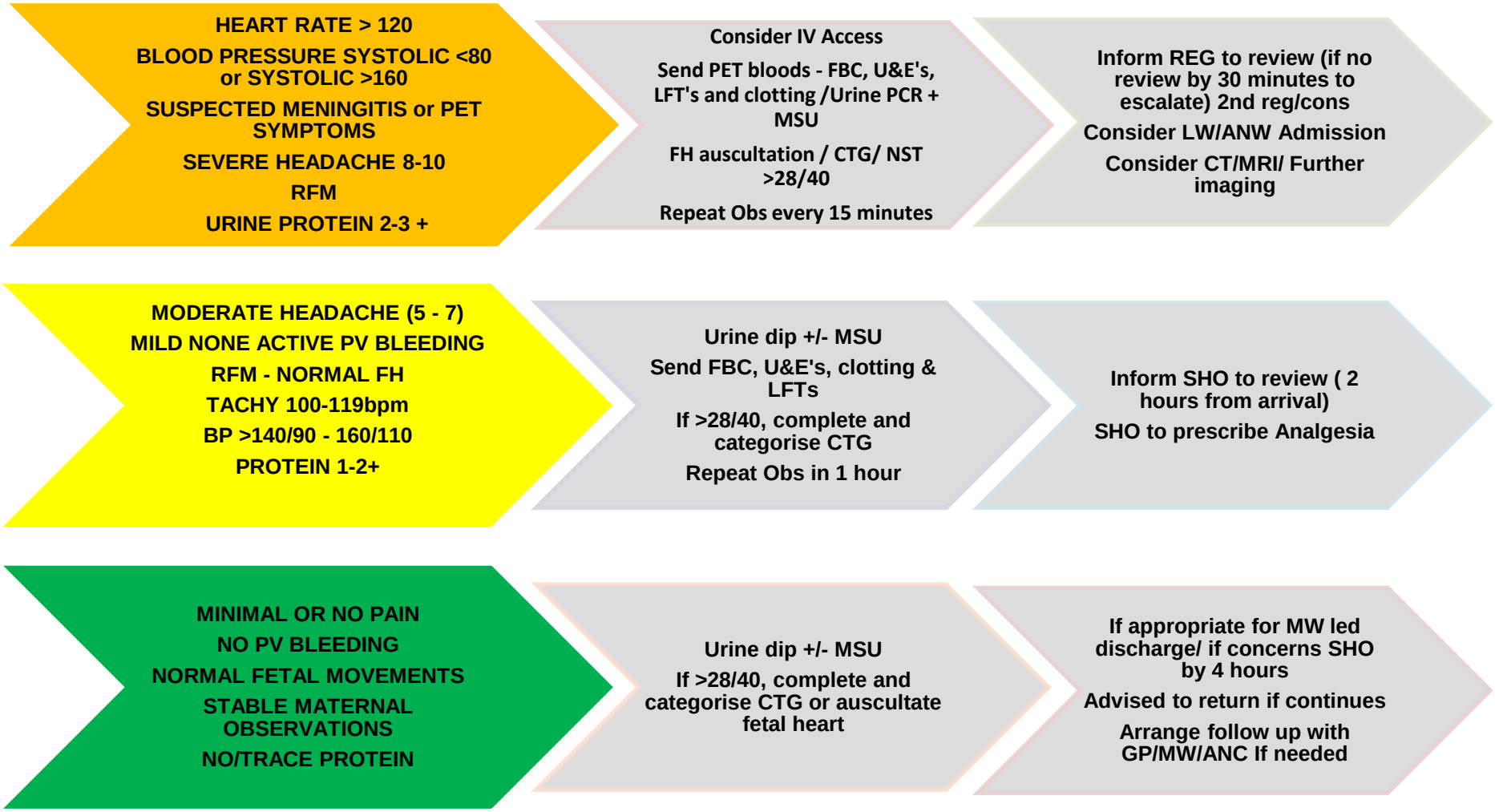
ABDOMINAL PAIN FLOWCHART



EARLY and Active LABOUR ASSESSMENT FLOWCHART



HEADACHE IN PREGNANCY FLOWCHART



RAISED BLOOD PRESSURE IN PREGNANCY FLOWCHART

SEVERE HEADACHE
SOB
VOMMITING
ABNORMAL MEWS 1XR 2XY
BP >160 SYSTOLIC OR >110 DIASTOLIC X 2 READINGS
PROTEINURIA 3+
FETAL HR <110BPM OR >160BPM
MODERATE/ CONTINUOUS PAIN
MODERATE ACTIVE BLEEDING

→ **Urgent PET bloods**
Consider IV access
MSU/PCR
CTG/NST >28/40 OR FH
Repeat obs 15 minutes
Follow Hypertension guideline

→ **Treat Hypertension following guideline**
For registrar review by 45 minutes from arrival - to reescalate if no review by 30 minutes

MILD/MODERATE HEADACHE
MILD PAIN
MILD PVB (NOT ACTIVE)
BP140/90
ALTERED MEWS 1 X Y
PROTEINURA 1-2+
RFM
NORMAL FH

→ **Urine dip, Urine PCR + MSU**
Send PET Bloods
CTG/NST >28/40
Repeat BP 15 minutes

→ **For SHO 2 hours from arrival**
To follow hypertension guideline
Consider treatment if 2 BP readings above 140/90
DAU twice weekly for BP check- Aim for 135/85mmg

BP <140/90
NO HEADACHE
NORMAL MEWS
NO /TRACE PROTEIN
NORMAL FH
NORMAL FM
MINIMAL OR NO PAIN

→ **Send MSU if positive urine dip**
FH
Repeat BP in 15 minutes if borderline

→ **If normal BP readings x 2**
No proteinuria can be discharged by mw for rouine care
If other concerns for SHO review by 4 hours

REDUCED FETAL MOVEMENTS IN PREGNANCY FLOWCHART

NO FH / UNABLE TO AUSCULATE (URGENT REVIEW BTY 15 MINUTES)
FH <110BPM >160BPM
NO FM PRIOR TO ATTENDING
2ND OR SUBSEQUENT RFM VISIT
MODERATE ACTIVE BLEEDING
RISK FOR SB/SGA/FGR
MODERATE ABDO PAIN
PRE EXISITING MEDICAL CONDITION

→ **USS by REG if cannot locate FH**
CTG/NST >28/40 by 15 minutes
Repeat obs 1 hour

→ **REG review by 30 minutes (45 from arrival)**
Consider LW/ANW admission

RFM OR ALTERED PATTERN
MILD ABDO PAIN
MILD PV BLEEDING (NOT ACTIVE)
NORMAL FH 110-160BPM
ALTERED MEWS 1X Y

→ **NST/CTG within 30 minutes if >28/40**
FH only if <28/40
If altered Mews repeat in 30 minutes

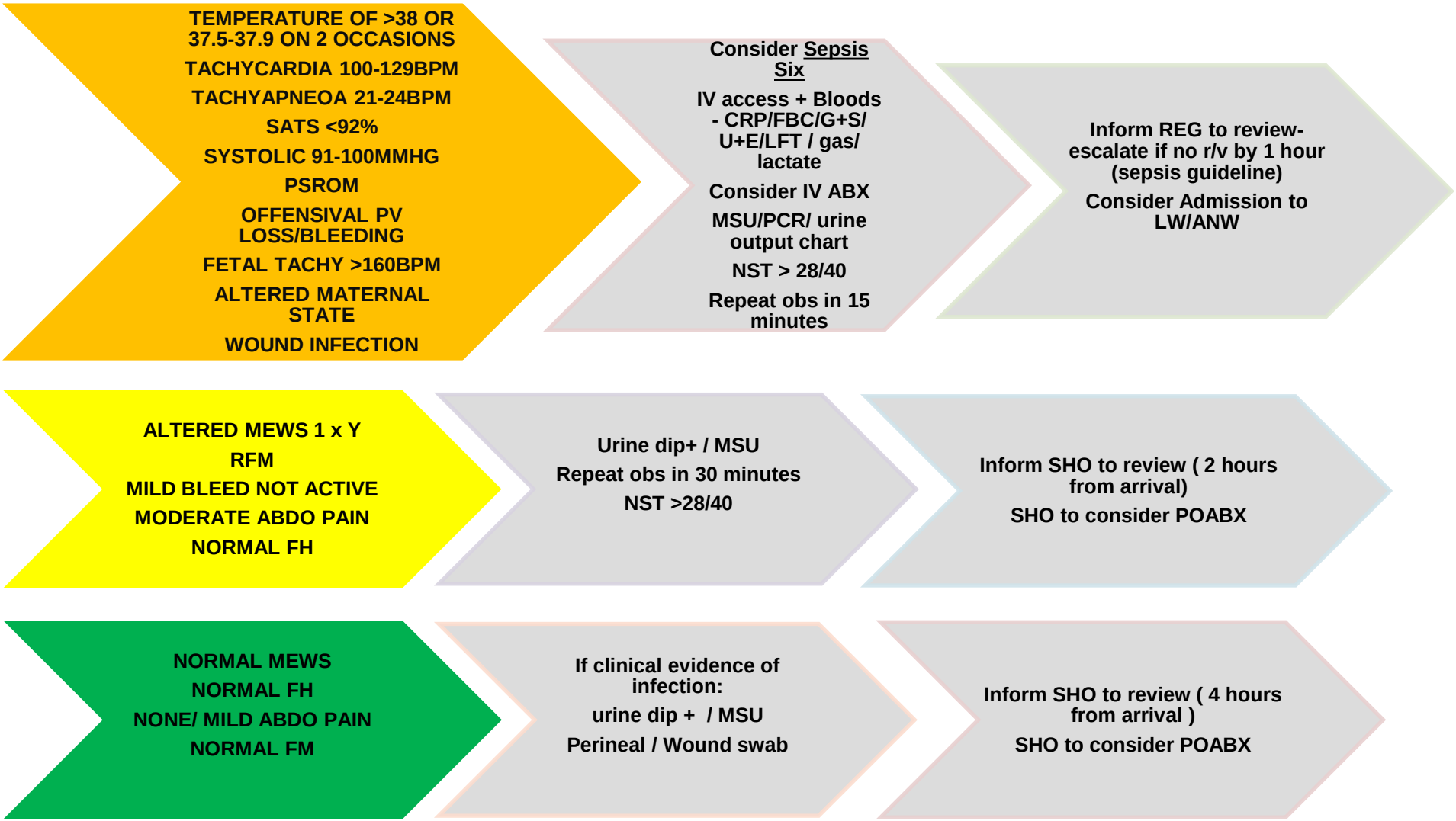
→ **SHO review by 2 hours from arrival**
MW discharge if FM return to usual pattern + normal NST at 1st episode less than 39/40

MINIMAL OR NO PAIN
NO PV BLEEDING
NORMAL FETAL MOVEMENTS ON ADMISSION
STABLE MATERNAL OBSERVATIONS
PATIENT TO BE SEEN WITHIN 4 HOURS

→ **Ausculate FH**

→ **MW Led discharge**

SEPSIS IN PREGNANCY FLOWCHART



*FOR ED REFERAL IF SOB SYMPTOMS ONLY
ie normal FM AND no PVB/ABDO PAIN *

SHORTNESS OF BREATH IN PREGNANCY FLOWCHART

HEART RATE > 120
BLOOD PRESSURE SYSTOLIC <80
SATS <92%
SUSPECTED PE / SEVERE CHEST PAIN
ACTIVE PV BLEED
SEVERE ABDO PAIN
RFM

→ **Consider IV Access**
Give high flow oxygen via face mask. Give inhaler/nebs if known asthma / peak flow
Bloods - FBC, U&E's, CRP, LFT's. Perfom ABG and ECG
FH/NST >28/40
Repeat Obs every 15 minutes

→ **Inform REG to review (escalate if no review by 30 minutes)**
co-ordinator for transfer to Labour Ward

PRODUCTIVE COUGH
ASSOCIATED MODERATE CHEST PAIN
ASSOCIATED PYREXIA
TACHYCARDIA 100-120BPM
RFM
NONE ACTIVE PVB
MODERATE ABDO PAIN

→ **Consider ECG**
Send FBC, U&E's, CRP. Troponin if chest pain
Consider Covid/flu swab
If >28/40, complete and categorise CTG
Repeat Obs in 1 hour

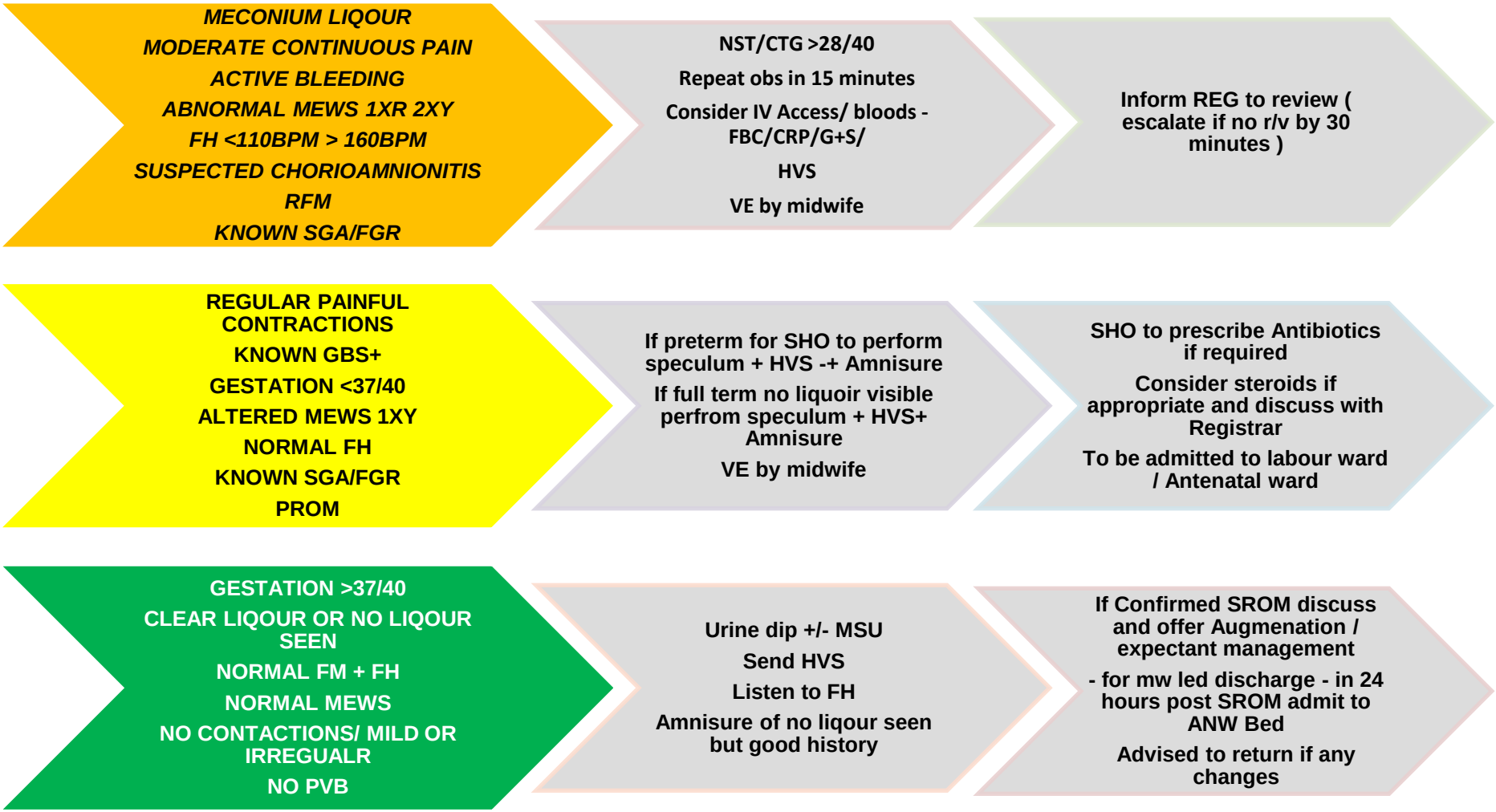
→ **Inform SHO of review (escalate if no reivew by 2 hours)**

MINIMAL OR NO PAIN
NORMAL FETAL MOVEMENTS
STABLE MATERNAL OBSERVATIONS

→ **Send MSU if positive urine dip**
If >28/40, complete and categorise CTG

→ **Advise to return if continues**
Arrange follow up with GP/MW/ANC if needed
SHO to reivew by 4 hours if wishes

SPONTANEOUS RUPTURE OF MEMBRANES FLOWCHART



VAGINAL BLEEDING IN PREGNANCY FLOWCHART

ANY ACTIVE BLEEDING
ABNORMAL MEWS 1XR 1XY
NO FM
FH <110BPM > 160BPM
MODERATE/ CONTINUOUS PAIN

Consider IV access
Bloods - FBC, G+S, X Match, Clotting, kleihaur if RH NEG
Review placetnal site on USS
NST/CTG >28/40
Repeat Obs every 15 minutes
Keep Nil by mouth

Infom REG to review (escalate if no review by 30 mintues)
Consider admission to LW

MILD BLEEDING (NOT CURRENTLY ACTIVE)
MILD PAIN
ALTERED MEWS 1XY
RFM
NORMAL FH

Urine dip +/- MSU
Send FBC, clotting, G&S, Kleihaur if RH NEG
Send HVS if appropriate
NST/CTG >28/40
Repeat Obs in 1 hour

Inform SHO to review (escalate if no review by 2 hours)
Consider admission to ANW/LW

MINIMAL OR NO PAIN
BLEEDING SETTLED
NORMAL FETAL MOVEMENTS
STABLE MATERNAL OBSERVATIONS
NORMAL FH

Urine dip +/- MSU
Send HVS if appropriate
If old evidence for NST/CTG >28/40
FH only if show

If no evidence on speculum and provoked bleed - for discharge
Advised to return if continues
Arrange follow up with GP/MW/ANC if needed
SHO to review 4 hours if required

CATEGORY OF URGENCY		
RED STAT – SEE CRITERIA		
AMBER	YELLOW	GREEN
TIME OF ONGOING MIDWIFERY CARE:		
15 MINUTES	1 HOUR	4 HOURS
Ongoing care includes bloods, VE, NST/CTG, Repeat observation (MSW), Review overall clinical picture		
TIME OF OBSTETRIC REVIEW FROM INITIAL MIDWIFE ASSESSMENT IF REQUIRED:		
15-45 MINUTES	2 HOURS	4 HOURS
Clinical judgement should always be used Escalate if no review within ideal time frame Upgrade urgency if clinical deterioration		

Appendix 4- Red and Amber categories

RED Category of Urgency

Obstetric Emergencies – Consider 2222 call

- Maternal collapse/ arrest/ obstructed airway/ seizures
- Severe Sepsis – Altered level of consciousness/ confusion
- Major Haemorrhage
- Severe hypertension/ fulminating Eclampsia (BP >180/115)
- Fetal Bradycardia
- Cord Prolapse
- Constant Severe Abdominal pain/suspected abruption
- Abnormal Mews Flagging RED – see Sepsis guideline

If collapse/arrest – call 2222 for multidisciplinary team, commence treatment in triage and transfer when stable
For imminent birth and other RED flags transfer to labour ward STAT (intrapartum care may not require obstetric review)
Inform Senior obstetric team WHO, Anaesthetic team WHO and Midwife IN triage or on LWin charge of patient with RED priority, requires **Immediate** review

AMBER Category of Urgency

ONGOING MIDWIFERY CARE + REG review within *15 minutes:*

- PET/Clinically unwell
- BP >160/110
- No/RFM and unable to locate FH
- Threatened preterm labour TPTL or PROM <34/40
- Significant APH
- Postnatal – Suspected sepsis, secondary haemorrhage

Other AMBER clinical scenarios in Appendix 2 will have ongoing care by midwife within 15 minutes but require a obstetric review by 45 minutes from arrival
All RFM require NST/CTG to be started by 15 minutes – if other symptoms present, do not delay obstetric review whilst awaiting NST/CTG findings